

NAME	HEGDE ARCHANA	STUDY	05-04-2025 13:27:58
AGE/GENDER	40Y 6M 21D / F	UHID	700867295
ACC NO	70337284	MOD	CT,PT
REFERER	Dr. RADIATION ONCOLOGY	REPORT	07-04-2025 13:00:46

18F-FDG PET CT REPORT

Clinical Details: k/c/o ca rectum post chemotherapy. 18F-FDG PET CT was done for further evaluation.

Technique: Whole body images (vertex to mid-thigh) were acquired in 3-D mode 100 min after i.v. injection of 9.2 mCi of 18F-FDG using GE Discovery 600 (16 slice CT) scanner. FBS before tracer injection was 130 mg/dl. Water was used as negative oral contrast. IV contrast was used. Serum creatinine was 0.65 mg/dL. Reconstruction of the acquired data was performed so as to obtain fused PET-CT images in transaxial, coronal and sagittal views. Additional HRCT chest acquired.

Comparison: compared to previous PETCT dated 05.02.2025

FINDINGS:

Brain:

- Normal physiological tracer uptake seen involving neuro-parenchyma. No focal mass lesions / focal abnormal FDG uptake noted.

Head and Neck:

- No discrete enhancing mass lesion/focal abnormal FDG uptake is seen in the nasopharynx / oropharynx / hypopharynx.
- No significant FDG avid cervical lymph nodes.
- Both lobes of thyroid gland appear normal in size. No evidence of hypodense lesions or focal abnormal FDG uptake.

Breast and axilla:

- Breast parenchyma appears unremarkable. No abnormal metabolic active focal lesions seen.
- Few sub-cm non FDG avid bilateral axillary lymph nodes with preserved shape & hilum seen.

Thorax:

- Bilateral lung parenchyma appears unremarkable. Fibrotic changes noted in the lateral segment of right middle lobe.
- Stable- Non FDG avid -3.8mm left oblique para-fissural nodule noted in the posterior segment of left upper lobe. No significant FDG avid soft tissue nodules or focal abnormal FDG uptake noted. No evidence of pleural effusion or pleural thickening noted on either side.
- No significant FDG avid mediastinal lymph nodes.
- Esophagus and GE junction appear unremarkable.

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Abdomen & pelvis:

- **BOWEL:** Complete interval resolution of size and FDG uptake in previously asymmetric FDG avid enhancing wall thickening seen involving the upper rectum and recto sigmoid junction (Previously involving anterior and lateral wall from 8 to 3 O clock in position, extending for a length of 6.9cm with single maximum wall thickness measures 2.2cm with SUV max of 17.73, -9 cm away from the anal verge).
- Interval resolution of previously mild mesorectal fat stranding noted with multiple mild FDG avid enlarged mesorectal lymph nodes (previously measuring- 7-8mm in SAD with SUV max of 2.64).
- No evidence of dilatation of proximal bowel loops.
- Stomach, 'C' loop of duodenum and rest of the visualised small / large bowel loops appear normal.
- Liver appears normal in size. A tiny haemangioma noted in the segment 7 measuring- 3mm. No evidence of hypoattenuating lesions or focal abnormal FDG uptake in the liver. No evidence of IHBRD. Gall bladder appears unremarkable with no evidence of calculi.
- Spleen appears normal in size. No focal hypodense lesion or abnormal FDG uptake is seen.
- Pancreas appears normal in size, morphology with no mass lesion/focal abnormal FDG uptake. No dilatation seen in the main pancreatic duct.
- Bilateral adrenal glands appear normal in size with no focal nodules or focal FDG uptake.
- Bilateral kidneys appear normal in size, shape and position. No evidence of renal calculi.
- No significant FDG avid abdominal / pelvic/ retroperitoneal lymph nodes are noted.
- **Uterus appears bulky in size with few fibroids as described:**
 - Hetero-hypodense subserosal fibroid noted in the anterior myometrium measuring -4.0 x 3.4cm (previously 3.8x2.9cm).
 - Interval reduction in the size of intensely FDG avid intramural fibroid showing uniform enhancement in the left lateral wall measuring - 2.7 x 2.3cm, SUVmax 14.3 (previously 3.5 x 2.4cm with SUV max of 14.37).
 - Interval resolution of previously FDG avid soft tissue density bridging tissue with the thickened rectal mass (previously -8 x 16mm with SUV max of 7.49).
- Ovaries appear normal. No FDG avid adnexal mass lesion seen.
- Trace POD free fluid noted.
- Few sub-cm non FDG avid bilateral inguinal lymph nodes with preserved shape & hilum seen.

Musculo-Skeletal system:

- No discrete FDG avid lytic / sclerotic lesions seen in visualized bones.

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IMPRESSION: compared to previous PETCT dated 05.02.2025, current scan shows

- Complete resolution of previously noted malignant FDG avid wall thickening involving the upper rectum and recto sigmoid junction (as described above) with resolution of surrounding locoregional nodes as described.
- Bulky uterus with fibroids-
 - Interval reduction in the size of one fibroid in the left lateral wall shows intense FDG uptake and uniform enhancement.
 - Interval resolution of previously FDG avid soft tissue density bridging tissue with the thickened rectal mass
- No evidence of distal metastasis.



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